Informative dropout and trial-design choice in aggregated N-of-1 biomarker-validation trials: a paired simulation at one parameter point

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Abstract

Background. Participants leave aggregated N-of-1 trials, and their reasons can depend on how their symptoms change. We asked how dropout affects the test of a biomarker-by-treatment interaction in three designs.

Methods. We simulated open-label titration with blinded discontinuation (OL+BDC), a two-period crossover (CO) and the Hybrid design at one parameter point, with $N = 70$ in each. The biomarker shifted the drug-response mean at on-drug visits, with true interaction -2.25. Seven dropout conditions were applied to the same 1,000 complete datasets per design. They were no dropout, completely random dropout and magnitude-driven dropout at 25 and 40 percent, and three directional mechanisms at 25 percent. We fitted the mixed model at analysis half-lives of 0.5 week and 0, with model-based and cluster-robust (CR2) tests. A null regime had 500 datasets per design.

Results. The 0.5-week analysis half-life biased the interaction estimate by -0.24 in OL+BDC and -0.18 in the Hybrid design (MCSE 0.02). A binary exposure indicator, which matches the generating interaction, removed that bias. Without dropout the empirical standard deviation of the estimate was 0.548 (Hybrid), 0.560 (OL+BDC) and 0.747 (CO). Dropout at 25 percent raised the standard error by a factor of 1.081 to 1.091 under completely random dropout. At

40 percent magnitude-driven dropout the factor was 1.132 to 1.163, nearly the same in all three designs. CO lost more absolute power, mostly because it started lower on its power curve. Improvement-driven dropout attenuated the estimate by 0.039 to 0.149 points (paired MCSE at most 0.012), most in CO. Dropout did not change test size. The model-based test rejected in 0.014 to 0.024 of null Hybrid datasets. The CR2 test was calibrated: in 5,000 independent null OL+BDC datasets it rejected in 0.051 (MCSE 0.003).

Conclusions. At this parameter point the Hybrid and OL+BDC designs carried the same information and lost it to dropout at the same rate. CO needed about 1.9 times the sample for the same precision. The analysis half-life must match the generating interaction. The direction of dropout mattered for bias, not for precision.

1 Introduction

1.1 The question

Aggregated N-of-1 trials pool many single-participant series of on-drug and off-drug periods into one mixed model[1]. A predictive-biomarker trial of this kind tests whether a baseline biomarker modifies the drug effect. The target is the biomarker-by-treatment interaction $\beta_{bm:D}$. Participants leave such trials, and their reasons can depend on how their symptoms are changing. We ask whether three candidate designs differ in how much power to detect $\beta_{bm:D}$ they lose to dropout, and whether the answer depends on the dropout mechanism or only on its rate.

The three designs are open-label titration followed by blinded discontinuation (OL+BDC), the traditional two-period crossover (CO), and the Hybrid design of[2]. The Hybrid design is open-label titration, then blinded discontinuation, then a brief crossover. A fourth design, open label throughout (OL), has no off-drug visits. It cannot separate the drug response from the placebo response and the natural history, so it carries no information on $\beta_{bm:D}$. We exclude it from the comparison for that reason (Section 3).

1.2 Prior work

[2] introduced the Hybrid design and compared it with OL, OL+BDC and CO by simulation. Their censoring filter assigned each simulated participant a per-unit-time dropout probability equal to a flat hazard plus a term in the square of the symptom change since baseline, shifted by 100. They describe this as a probability that is higher for participants who worsen or remain symptomatic and lower for participants who benefit (Methods). They report that increased censoring lowered power in every design, with a faster decline for OL+BDC because of participant loss before the blinded phase (Results, Figure 4A). In their comparison the Hybrid design had higher power than OL and OL+BDC and slightly lower power than CO without carryover. Its power fell faster than that of CO once carryover was introduced. Type I error rose with censoring, most for OL+BDC (Figure 6). Under the null, the interaction estimate was biased toward negative values for most censoring patterns in OL+BDC

and for several in CO (Figure 7A). Against an uncensored reference, censoring changed the estimate only in the OL design (Figure 7B). Their 'happy accident' refers to trial cost. Participants with no response, or with a response clearly lost on placebo, tend to leave before the later crossover blocks, so those blocks are spent on participants whose earlier phases were inconclusive (Discussion). It is not a statement about power.

The present study does not reproduce those results. It uses different correlation parameters, carryover, analysis model and dropout hazards (Section 7.2).

[3] studied two-period crossover designs under non-ignorable dropout. They found the conventional AB/BA design robust under a missing-at-random analysis, and they identified a sequence set that is preferable when dropout depends on the within-participant difference between periods.[4] compared aggregated N-of-1 designs with parallel and crossover trials by simulation, including selection bias. N-of-1 reporting guidance asks that dropout be reported[5], and reviews of published N-of-1 studies assess how completely trials report their methods[6,7]. We did not find a simulation that compares OL+BDC, CO and the Hybrid design on paired data under several dropout mechanisms.

The missing-data background is standard.[8] defined data missing completely at random (MCAR), missing at random (MAR) and missing not at random (MNAR)[9]. Selection models[10], pattern-mixture models[11,12] and shared-parameter models[13,14] describe dropout that depends on unobserved values. Likelihood-based mixed models are the usual primary analysis when dropout is plausibly MAR[15–19]. Sensitivity analyses vary the departure from MAR through delta-adjusted or reference-based multiple imputation[20–24] and tipping-point displays[25]. The ICH E9(R1) addendum treats dropout as an intercurrent event to be handled through the estimand[26,27]. This paper uses only the first of these tools, the likelihood-based mixed model. Joint models for longitudinal and event-time data[28,29] and run-in selection[30] are related but are not used here.

1.3 What this paper contributes

We make three contributions. First, we define the simulated dropout mechanisms exactly and state their missing-data properties (Section 2). Second, we report a paired simulation in which each dropout condition is applied to the same complete datasets. Power losses, coefficient shifts and standard-error ratios are then paired differences with small Monte Carlo standard errors (MCSE). Third, we show that the analysis half-life must match the generating interaction. The historical analysis decays the exposure indicator at 0.5 week, while the generating interaction has no carryover. This mismatch biases the estimate for OL+BDC and the Hybrid design and changes the design ordering (Section 5.2).

The study covers one parameter point. It is exploratory. No analysis plan was registered in advance, and its findings should be confirmed in an independent study. Three components of the working protocol were not carried out: the sample-size, effect-size and carryover sweeps; a path-stratified decomposition of the dropout pattern in the Hybrid design; and analysis-stage sensitivity analyses under alternative missing-data assumptions. Section 7.3 lists what each would add.

2 Dropout mechanisms and their missing-data properties

2.1 The simulated hazards

All dropout is generated by the function `first_dropout()` in the study driver (`analysis/scripts/informative-dropout-by-design/20-paired-dropout.R`). The package function `censordata()` is not used (Section 2.2).

Let Y_{it} be the symptom score of participant i at post-baseline visit t and BL_i the baseline score. Higher scores are worse. The change from baseline is $\Delta_{it} = Y_{it} - \mathrm{BL}_i$, so a negative change is an improvement. The step change is $r_{it} = Y_{it} - Y_{i,t-1}$, with $Y_{i0} = \mathrm{BL}_i$. Within each randomization path and visit, let $\bar{\Delta}_t$ and s_t be the mean and standard deviation of Δ_{it} across the participants of that path, and define $\bar{r}_t$ and s_t^r in the same way. The raw hazard h_{it} of each condition is:

- MCAR: $h_{it} = 1$.
- Magnitude of change: $h_{it} = \min\{1, \beta_0 + \beta_1(\Delta_{it}/s_t)^2\}$ with $\beta_0 = 0.05$ and $\beta_1 = 0.5$. The change is scaled but not centered.
- Worsening-driven: $h_{it} = \exp\{\gamma(\Delta_{it} - \bar{\Delta}_t)/s_t\}$.
- Improvement-driven: $h_{it} = \exp\{-\gamma(\Delta_{it} - \bar{\Delta}_t)/s_t\}$.
- Symptom return: $h_{it} = \exp\{\gamma(r_{it} - \bar{r}_t)/s_t^r\}$.

We set $\gamma = \ln 3$. Under the three directional hazards the raw hazard triples for each standard deviation of the driving quantity.

The hazard applied at visit t is $\min(1, \alpha h_{it})$. The participant drops out at the first visit t with $U_{it} < \min(1, \alpha h_{it})$, where the U_{it} are independent uniform draws. That visit and all later visits are deleted. Baseline is always observed. The multiplier α is found by root finding on each realized path dataset. It solves

$$\frac{1}{n}\sum_i \Big[1 - \prod_t \{1 - \min(1, \alpha h_{it})\}\Big] = d,$$

where n is the number of participants on the path and d is the target fraction of participants who ever drop out (0.25 or 0.40). The achieved fraction therefore matches d in expectation by construction. It is not an outcome of the study.

The hazard is per visit, not per unit of time. Visits are 2.5 weeks apart throughout CO, 4 weeks apart in the open-label phases of OL+BDC and the Hybrid design, and 1 week apart in their blinded phases. A common target d therefore places dropout at different calendar times and phases in different designs. Section 5.1 reports where dropout fell.

The 'biased' condition of the earlier unpaired prototype is the magnitude-of-change hazard. It rises with $|\Delta_{it}|$ in either direction. The mean change is an improvement at every post-baseline visit, so a large $|\Delta_{it}|$ is more often a large improvement than a large worsening. That condition

is not worsening-driven, and we no longer label it so. The worsening-driven, improvement-driven and symptom-return hazards were added to separate the direction of selection from its magnitude.

2.2 Relation to `censordata()` and to the hazard of Hendrickson et al.

The package function `censordata()` combines a flat Bernoulli draw and a symptom-dependent Bernoulli draw. In the published code (commit 58b32a9) the symptom-dependent term is proportional to $(D + 100)^{e_{b1}}$, where D is the generated column `D_<visit>`. That column is the sum of the response components, which equals $\mathrm{BL}_i - Y_{it}$, the improvement, so the term increases with improvement.[2] describe the term as the change in symptoms since baseline shifted by 100, which makes dropout higher for participants who worsen. The code and the description disagree in sign. The package and its live implementations now use $(100 - D)^{e_{b1}}$, as described; the frozen reference implementation keeps the published form. Both draws are proportional to the interval since the previous visit, `t_wk`, so the parameter β_0 is a per-visit hazard rather than a total dropout rate. The present study does not call `censordata()`, so none of these points affects its results.

2.3 Missing-data classification

Under the MCAR condition dropout is independent of all outcomes. It is missing completely at random[8].

Under the four outcome-dependent conditions, dropout at visit t depends on Y_{it}, and Y_{it} is then deleted. Dropout therefore depends on an unobserved value, and the data are MNAR. This is the selection-model form of[10]. Two features weaken the practical consequence. First, the serial correlation of the generating process is $0.7^{|w_t - w_s|}$ within each response component, where w_t is the calendar week. Y_{it} is therefore partly predicted by $Y_{i,t-1}$, which is observed. We expect the mechanism to behave nearly as MAR, but we did not quantify this. Second, the magnitude-of-change hazard is symmetric in the direction of change. It removes strongly improving and strongly worsening participants alike.

MAR alone does not make the likelihood-based analysis unbiased. Valid likelihood inference under MAR also requires a correctly specified model for the full data[9,18]. The fitted mean is linear in calendar time with a random intercept and a continuous-time first-order autoregressive (CAR(1)) residual. The generating mean is a sum of Gompertz curves with three autocorrelated components. MAR dropout can therefore bias $\hat{\beta}_{bm:D}$ here, and any bias we observe cannot be attributed to MNAR alone.

We did not simulate shared-parameter dropout[13,14]. In that mechanism the hazard depends on a participant's latent drug response, which is the quantity the biomarker predicts. It is the mechanism most likely to bias $\beta_{bm:D}$ and the one that the clinical motivation most directly suggests. Our symptom-return hazard depends on the observed off-drug rise in symptoms. That rise is larger for participants with a larger drug response, so the hazard is related to the latent response only through the observed outcome.

2.4 Estimand

We treat dropout as an intercurrent event[26]. The target is the hypothetical-strategy estimand: $\beta_{bm:D}$ as it would be had every participant completed the schedule. Its true value is fixed by the generating process (Section 4).

3 The designs and where dropout falls

Each design is a set of randomization paths. A path is a sequence of visits, each on drug or off drug. The interaction is identified by within-participant contrasts between on-drug and off-drug visits. A dropout removes the remaining visits of a path, and with them any contrast they carry.

OL+BDC has eight visits at weeks 4, 8, 12, 16, 17, 18, 19 and 20. Both of its two paths are on drug through week 16. One path switches to placebo after week 18 and the other after week 17. The off-drug visits are therefore the last two or three, and a dropout before them removes the only off-drug information for that participant.

CO has eight visits 2.5 weeks apart. One path is on drug for the first four visits and on placebo for the last four. The other path is the reverse. Each participant contributes four visits on each side of the switch, and a dropout in the second period removes part of one side.

The Hybrid design has visits at weeks 4, 8, 9, 10, 11, 12, 16 and 20. All four paths are on drug through week 9. Two paths switch to placebo after week 9 and two after week 10. The last two visits, at weeks 16 and 20, are a drug-placebo crossover in either order. Off-drug visits occur in the blinded phase and in the crossover.

The OL design is on drug at every visit. The package analysis `lme_analysis()` detects that no participant varies in exposure after baseline and then fits `Sx ~ bm + t + bm:t`. The coefficient it tests is the biomarker-by-time slope, not $\beta_{bm:D}$. The earlier prototype reported that slope test in its OL row. Its rejection rate, 0.086 to 0.112, was the power of a different test. It was neither a test size nor a measure of information on the interaction. A model with `bm:Dbc` and the baseline row would be estimable for OL, but its interaction is not separable from the natural-history and placebo components. We therefore exclude OL.

These descriptions say where each design's off-drug information lies. They are not tested hypotheses about which design loses most. The results in Section 5 address that question directly.

4 Simulation design

We report the study in the ADEMP structure of[31].

Aims. To compare OL+BDC, CO and the Hybrid design on the power to detect $\beta_{bm:D}$, its bias and the calibration of its test, without dropout and under six dropout conditions, at one parameter point, with paired comparisons across conditions.

Data-generating mechanism. Data were generated by the package function `generateData()` under the mean-shift encoding (`dgp_architecture = 'mean_moderation'`, `moderation_scaling = 'constant'`). The biomarker B_i has mean 0 and standard deviation 1, so it coincides with its standardized value b_i. At every on-drug visit, the drug-response component is shifted by $\beta_{bm} b_i \sigma_{BR}$ with $\beta_{bm} = 0.45$ and $\sigma_{BR} = 5$. Off-drug visits receive no shift, so the shift has no carryover. The mean drug response itself carries over after discontinuation with half-life $t_{1/2} = 0.5$ week, decaying from the last on-drug mean. The three response components (drug response, placebo response, natural history) follow modified Gompertz means with the reference parameters of the driver. Each component has first-order autoregressive correlation $\rho = 0.7$ per week. Components are correlated at 0.1 at the same visit and 0.05 across visits. Baseline severity has mean 70 and standard deviation 10. Every covariance matrix was positive definite, and the driver refuses any matrix that is not.

The components reduce symptoms, so a larger drug response lowers Y_{it}. The interaction is therefore negative. Its true value is $\beta_{bm:D} = -\beta_{bm}\sigma_{BR}/\sigma_{bm} = -2.25$ points per biomarker unit, under binary drug exposure. Under the null regime $\beta_{bm} = 0$ and the true value is 0.

Every design has a total of $N = 70$ participants, allocated across its paths: 35 on each path of OL+BDC and CO, and 18, 18, 17 and 17 on the four Hybrid paths.

Pairing. For each design and replicate, one complete dataset and one matrix of uniform draws U_{it} were generated. Each of the seven conditions was applied to that dataset with those draws. The conditions therefore differ only in the hazard. The null regime used its own seeds and was paired across conditions in the same way. Datasets were not shared across designs, so between-design comparisons are unpaired.

Estimand. $\beta_{bm:D}$ under the hypothetical strategy, with true value -2.25 (alternative) or 0 (null).

Methods. Each post-dropout dataset was analyzed with

$$Y_{it} = \beta_0 + \beta_B B_i + \beta_T w_t + \beta_D D_{bc,it} + \beta_{bm:D} B_i D_{bc,it} + \beta_\eta \eta_t + u_i + \varepsilon_{it},$$

with a participant random intercept u_i and CAR(1) residual correlation in calendar weeks, fitted by restricted maximum likelihood with `nlme::lme`. The design expectancy η_t enters for OL+BDC and the Hybrid design. CO has a constant post-baseline expectancy, so the term is omitted there, as in `lme_analysis()`. The exposure indicator $D_{bc,it}$ is 1 on drug, 0 before the first exposure, and $2^{-t_{sd}/t^a_{1/2}}$ off drug after exposure, where t_{sd} is the time since discontinuation. We used two analysis half-lives:

- $t^a_{1/2} = 0.5$ week (analysis `ta0.5`): the historical specification, equal to the half-life of the mean drug response. It reproduces `lme_analysis()` exactly. We checked this on one dataset per design and obtained identical coefficients, standard errors and p-values.
- $t^a_{1/2} = 0$ (analysis `ta0`): binary drug exposure. This matches the generating interaction, which has no carryover.

Each fit was tested two ways. The model-based test uses the `nlme` standard error and degrees of freedom. The CR2 test uses the bias-reduced cluster-robust variance estimator[32] with

Satterthwaite degrees of freedom (`clubSandwich::coef_test`), as in the project's carryover-sensitivity driver. If the CAR(1) fit failed, the driver refitted without the residual correlation and recorded the fallback.

Performance measures. For each design, condition, analysis and test we report the rejection rate at $\alpha = 0.05$ (power under the alternative, size under the null), bias against the true value, the empirical standard deviation of $\hat{\beta}_{bm:D}$, the calibration ratio κ and the coverage of the 95 percent confidence interval. κ is the mean standard error divided by the empirical standard deviation of the estimates, and $\kappa > 1$ indicates a conservative test. Against the no-dropout condition on the same datasets we report the paired power loss, the paired shift in $\hat{\beta}_{bm:D}$ and the mean ratio of standard errors. Each carries its paired MCSE, the standard deviation of the per-replicate difference divided by the square root of the number of replicates. The MCSE of a rejection rate is $\sqrt{\hat{\pi}(1-\hat{\pi})/n_{\text{reps}}}$.

Replicates and seeds. The alternative regime has 1,000 replicates per design and the null regime 500. The seed of each design and replicate is `set.seed(20261029 + 100000 * d + r)`, where d is the design index (1 OL+BDC, 2 CO, 3 Hybrid) and r the replicate. The null regime adds 50,000. Replicates ran in parallel with `parallel::mclapply` on 8 workers, with single-threaded linear algebra.

5 Results

Every fit converged (100 percent of fits in every cell), and every fit used the CAR(1) residual (100 percent). No fallback fit occurred.

5.1 Dropout achieved and its direction

We first checked where dropout fell and whom it removed. Table 1 gives the results for the alternative regime. The fraction of participants who ever dropped out matched its target, as the calibration guarantees. The fraction of post-baseline observations lost was smaller, about 0.13 to 0.15 at the 25 percent target and 0.22 to 0.24 at 40 percent.

Where dropout fell differed by design. Dropouts occurred at an off-drug visit in 0.24 to 0.29 of cases in OL+BDC, 0.38 to 0.42 in the Hybrid design and 0.45 to 0.51 in CO. These fractions follow the share of off-drug visits in each design.

The direction of selection matched the construction of each hazard. Under the magnitude-of-change hazard, participants who dropped out were slightly more improved than those who stayed. The correlation with mean improvement was 0.035 to 0.080. In large single samples of 5,000 participants per path, the mean change from baseline was negative at every post-baseline visit of every path, between −2.4 and −28.4 points. A large squared change is therefore usually a large improvement. The worsening-driven and improvement-driven hazards gave correlations near -0.28 and 0.28. The symptom-return hazard was nearly unrelated to mean improvement (-0.029 to -0.019). Under the alternative, improvement-driven dropout

Table 1: Dropout achieved under the alternative, mean over replicates. "Obs. lost": fraction of post-baseline observations deleted. "At off-drug visit": fraction of dropouts whose first missing visit was off drug. The correlations are between the indicator of ever dropping out and the participant's mean improvement from baseline over the complete schedule, or the biomarker (MCSE of each correlation at most 0.006).

Design	Condition	Ever dropped	Obs. lost	At off-drug visit	Cor. with improvement	C
OL+BDC	MCAR 25%	0.252	0.148	0.278	-0.002	(
OL+BDC	Magnitude 25%	0.252	0.146	0.265	0.035	(
OL+BDC	Magnitude 40%	0.400	0.239	0.246	0.045	(
OL+BDC	Worsening 25%	0.250	0.148	0.272	-0.288	-
OL+BDC	Improvement 25%	0.252	0.150	0.269	0.290	(
OL+BDC	Symptom return 25%	0.253	0.148	0.285	-0.029	(
CO	MCAR 25%	0.252	0.148	0.497	0.000	-
CO	Magnitude 25%	0.250	0.135	0.448	0.060	(
CO	Magnitude 40%	0.400	0.223	0.454	0.080	(
CO	Worsening 25%	0.252	0.148	0.508	-0.262	-
CO	Improvement 25%	0.250	0.147	0.502	0.264	(
CO	Symptom return 25%	0.250	0.146	0.504	-0.024	-
Hybrid	MCAR 25%	0.249	0.146	0.419	0.003	(
Hybrid	Magnitude 25%	0.250	0.145	0.407	0.056	(
Hybrid	Magnitude 40%	0.398	0.238	0.387	0.070	(
Hybrid	Worsening 25%	0.248	0.146	0.409	-0.276	-
Hybrid	Improvement 25%	0.251	0.148	0.407	0.279	(
Hybrid	Symptom return 25%	0.247	0.144	0.420	-0.019	(

Table 2: Performance without dropout. Bias (MCSE) against the true value −2.25. Emp. SD: empirical standard deviation of the estimates. MB: model-based test. CR2: cluster-robust test with Satterthwaite degrees of freedom. Cover: coverage of the 95 percent interval. Size: rejection rate in the null regime. MCSE of power at most 0.011; of size at most 0.013.

Design	$t^a_{1/2}$	Bias	Emp. SD	κ MB	κ CR2	Cover MB	Cover CR2	Power MB	Power CR2	Size MB	Size CR2
OL+BDC	0.5	-0.238 (0.019)	0.615	1.16	0.96	0.967	0.932	0.959	0.974	0.048	0.090
OL+BDC	0	0.012 (0.018)	0.560	1.13	0.96	0.972	0.946	0.956	0.973	0.044	0.080
CO	0.5	-0.011 (0.024)	0.752	0.97	0.96	0.942	0.944	0.874	0.850	0.052	0.044
CO	0	0.001 (0.024)	0.747	0.97	0.96	0.942	0.944	0.872	0.852	0.052	0.046
Hybrid	0.5	-0.183 (0.019)	0.606	1.08	0.96	0.964	0.936	0.971	0.974	0.022	0.042
Hybrid	0	0.021 (0.017)	0.548	1.07	0.97	0.970	0.949	0.972	0.978	0.018	0.052

removed high-biomarker participants more often (correlation 0.068 to 0.103), because those participants had the larger drug response. Worsening-driven dropout did the reverse.

Because α was calibrated on each path separately, every path lost the same expected fraction of participants. The per-path fractions stored by the driver confirm this. A comparison of dropout shares across the Hybrid paths, the quantity a path decomposition would need, is therefore fixed by construction in this study and carries no information.

5.2 Bias, calibration and the analysis half-life

We next compared the two analysis half-lives without dropout. Table 2 gives the results. With the historical half-life of 0.5 week, the interaction estimate was biased by -0.238 (0.019) in OL+BDC and -0.183 (0.019) in the Hybrid design, on a true value of −2.25. These are 11 and 8 percent of the true value, away from zero. The cause is the mismatch between the analysis and the generating process. The analysis gives off-drug visits after discontinuation an exposure weight of $2^{-t_{sd}/0.5}$, while the generating shift is zero at those visits. The bias in CO was -0.011 (0.024). CO has few off-drug visits soon after a discontinuation, because its visits are 2.5 weeks apart. With a binary exposure indicator, the bias was 0.012 (0.018) in OL+BDC, 0.021 (0.017) in the Hybrid design and 0.001 (0.024) in CO.

The mismatch did not change power materially. The historical analysis inflated both the estimate and its standard error. Its empirical standard deviation in OL+BDC was 0.615 against 0.560 for the binary analysis. Model-based power was 0.959 and 0.956. The design ordering in power did not depend on the half-life. From here on we report the binary analysis, which is unbiased for this generating process.

The two tests were calibrated differently by design. The model-based test was conservative in the Hybrid design. Its κ was 1.07 and its null rejection rate 0.018 (0.006). It was close to nominal size in OL+BDC and CO, although its κ in OL+BDC under the alternative was 1.13. The CR2 test was close to nominal in the Hybrid design and CO. In OL+BDC it rejected in 0.080 (0.012) of the 500 null datasets of this run. That figure is a Monte Carlo excursion of these 500 datasets, not a property of the test. The same driver applied to 5,000 independent null datasets gave 0.051 (0.003) at $N = 70$ and 0.048 (0.003) at $N = 35$, with a CR2 standard error 0.98 times the empirical standard deviation (`analysis/scripts/cr2-olbdc-check/`). The model-based test was conservative there, rejecting in 0.027. The seven dropout conditions

Table 3: Power (MCSE) of the binary analysis ($t^a_{1/2} = 0$) by design, test and dropout condition, 1,000 paired datasets per design. MB: model-based. CR2: cluster-robust with Satterthwaite degrees of freedom.

Design	Test	None	MCAR 25%	Magnitude 25%	Magnitude 40%	Worsening 25%	Improvement 25%	Symptom return 25%
OL+BDC	MB	0.956 (0.006)	0.922 (0.008)	0.922 (0.008)	0.890 (0.010)	0.919 (0.009)	0.907 (0.009)	0.901 (0.009)
CO	MB	0.872 (0.011)	0.806 (0.013)	0.801 (0.013)	0.761 (0.013)	0.823 (0.012)	0.768 (0.013)	0.783 (0.013)
Hybrid	MB	0.972 (0.005)	0.940 (0.008)	0.937 (0.008)	0.918 (0.009)	0.934 (0.008)	0.935 (0.008)	0.942 (0.007)
OL+BDC	CR2	0.973 (0.005)	0.950 (0.007)	0.951 (0.007)	0.919 (0.009)	0.945 (0.007)	0.934 (0.008)	0.937 (0.008)
CO	CR2	0.852 (0.011)	0.774 (0.013)	0.774 (0.013)	0.720 (0.014)	0.782 (0.013)	0.747 (0.014)	0.765 (0.013)
Hybrid	CR2	0.978 (0.005)	0.953 (0.007)	0.948 (0.007)	0.932 (0.008)	0.936 (0.008)	0.947 (0.007)	0.954 (0.007)

of this run reuse the same 500 null datasets per design, so their null rejection rates are not independent replications. CR2 was within two MCSE of nominal in every design. The model-based test was conservative in the Hybrid design and OL+BDC. Comparisons of power across designs under the model-based test therefore mix information with test calibration. For that reason we also compare designs by the empirical standard deviation of the estimate, which does not depend on the test (Section 5.5).

Dropout did not change the size of either test. Across the six dropout conditions, the paired change in the null rejection rate ranged from -0.018 to 0.014, with paired MCSE at most 0.011.

5.3 Power by design and dropout condition

Table 3 gives power for the binary analysis under both tests. Hybrid and OL+BDC had model-based power of 0.972 and 0.956 without dropout. CO had 0.872. CO had the lowest power in every condition under both tests. Section 5.5 compares the Hybrid design with OL+BDC.

5.4 Paired losses, coefficient shifts and standard-error ratios

Table 4 gives the paired comparisons against the no-dropout condition on the same datasets. Pairing reduced the MCSE of a power loss by a factor of about 1.5 relative to independent cells.

Three results follow from the table.

First, the rate of dropout set the loss of precision, and the design did not. At 25 percent MCAR dropout the standard error rose by a factor of 1.081 to 1.091 across designs. At 40 percent magnitude-driven dropout it rose by 1.132 to 1.163. The paired MCSE of each ratio was at most 0.002. The 25 percent conditions of different mechanisms gave ratios within 0.028 of each other.

Second, the mechanism changed the estimate. MCAR, magnitude-driven and worsening-driven dropout moved the estimate by at most 0.031 in OL+BDC and CO. In the Hybrid design worsening-driven dropout moved it by 0.043 (0.008). Improvement-driven dropout moved the estimate toward zero by 0.149 (0.012) in CO, 0.098 (0.009) in OL+BDC and 0.039 (0.008) in the Hybrid design. Symptom-return dropout moved it by 0.044 to 0.065.

Table 4: Paired comparison of each dropout condition with no dropout, binary analysis. Loss: power without dropout minus power with it (paired MCSE). Shift: mean change in the estimate (paired MCSE). A positive shift moves the estimate toward zero. SE ratio: mean ratio of model-based standard errors (paired MCSE). SD ratio: ratio of empirical standard deviations.

Design	Condition	Loss MB	Loss CR2	Shift	SE ratio	SD ratio
OL+BDC	MCAR 25%	0.034 (0.007)	0.023 (0.006)	-0.018 (0.008)	1.091 (0.001)	1.060
OL+BDC	Magnitude 25%	0.034 (0.007)	0.022 (0.006)	-0.013 (0.008)	1.093 (0.001)	1.062
OL+BDC	Magnitude 40%	0.066 (0.009)	0.054 (0.008)	-0.017 (0.010)	1.163 (0.002)	1.121
OL+BDC	Worsening 25%	0.037 (0.007)	0.028 (0.006)	0.004 (0.008)	1.082 (0.001)	1.072
OL+BDC	Improvement 25%	0.049 (0.008)	0.039 (0.007)	0.098 (0.009)	1.088 (0.001)	1.024
OL+BDC	Symptom return 25%	0.055 (0.008)	0.036 (0.006)	0.059 (0.008)	1.075 (0.001)	1.065
CO	MCAR 25%	0.066 (0.010)	0.078 (0.010)	0.002 (0.010)	1.081 (0.001)	1.085
CO	Magnitude 25%	0.071 (0.010)	0.078 (0.010)	0.016 (0.010)	1.075 (0.001)	1.072
CO	Magnitude 40%	0.111 (0.011)	0.132 (0.013)	0.031 (0.013)	1.132 (0.002)	1.128
CO	Worsening 25%	0.049 (0.010)	0.070 (0.010)	0.019 (0.010)	1.072 (0.001)	1.086
CO	Improvement 25%	0.104 (0.011)	0.105 (0.012)	0.149 (0.012)	1.065 (0.001)	1.060
CO	Symptom return 25%	0.089 (0.011)	0.087 (0.011)	0.065 (0.011)	1.075 (0.001)	1.056
Hybrid	MCAR 25%	0.032 (0.007)	0.025 (0.006)	-0.001 (0.007)	1.084 (0.001)	1.067
Hybrid	Magnitude 25%	0.035 (0.006)	0.030 (0.006)	-0.006 (0.007)	1.085 (0.001)	1.076
Hybrid	Magnitude 40%	0.054 (0.008)	0.046 (0.007)	-0.018 (0.010)	1.150 (0.002)	1.109
Hybrid	Worsening 25%	0.038 (0.007)	0.042 (0.007)	0.043 (0.008)	1.080 (0.001)	1.063
Hybrid	Improvement 25%	0.037 (0.007)	0.031 (0.006)	0.039 (0.008)	1.077 (0.001)	1.070
Hybrid	Symptom return 25%	0.030 (0.006)	0.024 (0.006)	0.044 (0.007)	1.073 (0.001)	1.055

Improvement-driven dropout removes high-biomarker participants, who have the largest drug response (Section 5.1). The observed data then understate the interaction. The largest shift, 0.149 in CO, is 7 percent of the true value. Coverage of the 95 percent interval stayed between 0.938 and 0.956 for the CR2 test in every cell, because the shifts were small relative to the standard error.

Third, CO lost more absolute power than the other two designs in every condition. At 40 percent magnitude-driven dropout, model-based power fell by 0.111 (0.011) in CO, 0.066 (0.009) in OL+BDC and 0.054 (0.008) in the Hybrid design. The difference between CO and the Hybrid design was 0.057 (0.014) ($z = 4.09$). Most of this difference follows from the starting power, not from a design-specific loss of information. The standard-error ratios were nearly equal across designs. A normal approximation predicts the loss from the no-dropout power π_0 and the standard-error ratio s as $\pi_0 - \Phi\{(\Phi^{-1}(\pi_0) + 1.96)/s - 1.96\}$.

For 40 percent magnitude-driven dropout it gives 0.091 for CO, 0.073 for OL+BDC and 0.052 for the Hybrid design. The observed losses were 0.111, 0.066 and 0.054. Under improvement-driven dropout the extra loss in CO also reflects the attenuation of its estimate.

5.5 Design comparison on precision

Without dropout, the empirical standard deviation of the binary estimate was 0.548 in the Hybrid design, 0.560 in OL+BDC and 0.747 in CO (MCSE at most 0.017). The relative efficiency of CO to the Hybrid design was $(0.548/0.747)^2 = 0.54$. CO would need about 1.9 times as many participants to estimate the interaction as precisely, if precision scales with N. The Hybrid and OL+BDC designs differed by 2 percent in standard deviation. Their power losses differed by at most 0.025 across conditions and tests. The largest $|z|$ for a difference

Table 5: What the paired cells support, binary analysis unless stated, model-based shifts. Shifts are toward zero.

Situation	Finding
No dropout or MCAR dropout	Hybrid and OL+BDC equal in precision. CO needs about 1.9 times the sample
Dropout driven by the size of symptom change	Standard error rises by a similar factor in all designs (1.132 to 1.163 at 40 percent)
Worsening-driven dropout	Shift at most 0.043; precision loss as for MCAR
Improvement-driven dropout	Estimate attenuated by 0.149 (CO) to 0.039 (Hybrid)
Dropout on symptom return	Estimate attenuated by 0.044 to 0.065
Choice of analysis half-life	Mismatch biased OL+BDC by -0.24 and Hybrid by -0.18

in their losses was 2.46 among the 12 comparisons, which is within the range expected from chance alone.

5.6 Comparison with the earlier unpaired prototype

An earlier version of this study used independent datasets for each cell, 500 replicates per cell, the historical analysis and the magnitude-of-change hazard under the label 'biased'. It reported power losses at 40 percent of 0.086 (CO), 0.058 (Hybrid) and 0.056 (OL+BDC), with MCSE near 0.02. The paired rerun with the same analysis gave 0.111 (0.011), 0.058 (0.008) and 0.065 (0.009). The two runs agree within their Monte Carlo error. The earlier run could not separate the CO loss from the others. The paired run separates it, and the standard-error ratios show that the separation reflects CO's lower starting power.

The earlier run also reported an OL row. That row tested the biomarker-by-time slope (Section 3), not the interaction, and is not carried forward.

6 Implications for design choice

Table 5 states what the simulated cells support. It applies to this parameter point only.

On these results the Hybrid design and OL+BDC are interchangeable for the interaction. The choice between them can rest on participant burden and feasibility. CO is less efficient for this estimand at this parameter point, and its estimate was the most sensitive to improvement-driven dropout. The analysis half-life should match the generating interaction. In practice the true form is unknown, and a data-chosen half-life is the project's provisional recommendation (paper 02). We did not evaluate it here.

7 Discussion

7.1 What the paired design changed

Applying every dropout condition to the same datasets turned each loss and shift into a paired difference. This made the CO loss separable from the others and showed why. On the

standard-error scale the three designs lost the same precision to dropout of a given rate. CO lost more absolute power because it started lower. The earlier unpaired prototype had read its 3-point difference in losses as evidence that CO is the least robust design. With a standard error of about 3 points, that difference was not resolved.

7.2 Why the results differ from Hendrickson et al.

[2] found the Hybrid design slightly less powerful than CO without carryover. Here CO was the least efficient design. The two studies differ in several ways that could explain this. The original DGP uses compound symmetry with a common within-component correlation of 0.8. We used first-order autoregressive correlation of 0.7 per week. The original DGP encodes the interaction as a correlation between the biomarker and the drug response. We used the mean-shift encoding. The present carryover half-life is 0.5 week. Hendrickson et al. report that even 0.1 week of carryover reduced the power of the Hybrid design sharply. Their dropout hazard is per unit time and built on the shifted squared change, and ours is per visit. Which of these accounts for the difference was not tested.

7.3 Limitations

- **One parameter point.** Sample size, effect size, carryover half-life and correlation parameters were fixed. The protocol's sweeps over $N \in \{35, 70, 100\}$ and $\beta_{bm} \in \{0, 0.3, 0.6\}$ were not run.
- **One encoding.** The interaction was generated by the mean-shift encoding only. Under the correlation encoding the interaction has a different exposure profile, and the results may differ.
- **No shared-parameter dropout.** The hazards depend on observed outcomes. A hazard tied to the latent drug response would be more directly informative about the biomarker.
- **No analysis-stage sensitivity.** Every condition was analyzed with the likelihood-based mixed model under an implicit MAR assumption. Pattern-mixture, delta-adjusted and reference-based analyses[11,12,20–22] and tipping-point displays[25] were not applied.
- **No path decomposition.** The dropout calibration fixes each path's dropout fraction, so the Hybrid paths cannot be compared on dropout share. A path-stratified analysis would need a calibration across the whole design.
- **Per-visit hazard.** Visit spacing differs by design, so equal targets do not mean equal hazards per week.
- **Calibrated rates.** The ever-dropout fraction is set by construction. The study compares mechanisms at equal rates. It does not predict how often participants will leave a given design.
- **Test calibration.** The model-based test was conservative in the Hybrid design and OL+BDC. Power comparisons across designs under that test carry this difference. The

null regime has 500 datasets per design, so single-cell null rejection rates carry an MCSE of about 0.01. The CR2 calibration rests on a separate 5,000-dataset check.

- **Platform.** The run was made on the host R session, not in the project container. Results are not draw-for-draw reproducible across the two.
- **Symptom-return mechanism.** Its hazard uses the step change between consecutive visits, which differ in spacing. It was only weakly related to mean improvement and is one of several possible operationalizations.

8 Conclusions

At one parameter point with $N = 70$, the Hybrid and OL+BDC designs estimated the biomarker-by-treatment interaction with the same precision and lost the same share of it to dropout. CO needed about 1.9 times the sample for the same precision. Its larger absolute power loss under dropout followed mainly from its lower starting power.

The rate of dropout set the loss of precision. The direction of dropout set the bias. Improvement-driven dropout attenuated the estimate by up to 7 percent of its true value. Worsening-driven, magnitude-driven and MCAR dropout changed it little. Dropout did not change test size.

An analysis half-life of 0.5 week biased the OL+BDC and Hybrid estimates by about 11 and 8 percent, because the generating interaction had no carryover. The analysis must match the exposure profile of the interaction, not only that of the mean drug response.

Glossary

- **Analysis `ta0.5`.** The historical analysis: the exposure indicator decays after discontinuation with analysis half-life $t^a_{1/2} = 0.5$ week. It reproduces the package function `lme_analysis()`.
- **Analysis `ta0`.** The same model with $t^a_{1/2} = 0$, so the exposure indicator is binary. It matches the generating interaction.
- **Calibration ratio κ.** The mean standard error divided by the empirical standard deviation of the estimates. Values above 1 indicate a conservative test.
- **CR2.** The bias-reduced cluster-robust variance estimator of Bell and McCaffrey, clustered on participant, with Satterthwaite degrees of freedom.
- **CO.** The traditional two-period crossover: four visits on one treatment, then four on the other, in randomized order.
- **Dropout conditions.** `none`; `MCAR_25` (constant hazard, 25 percent ever drop out); `magnitude_25` and `magnitude_40` (hazard rising in the squared, uncentered, scaled change from baseline; stored in the earlier prototype as `biased_25` and `biased_40`); `worsening_25` and `improvement_25` (hazard rising or falling in the centered, scaled change from baseline); `return_25` (hazard rising in the centered, scaled change since the previous visit). Section 2 gives the formulas.

- **Hybrid design.** Open-label titration, then blinded discontinuation, then a brief two-visit crossover, with four randomization paths[2].
- **Mean-shift encoding.** The data-generating form in which the biomarker shifts the drug-response mean at on-drug visits by $\beta_{bm} b_i \sigma_{BR}$ (package value `dgp_architecture = 'mean_moderation'`).
- **OL, OL+BDC.** Open label throughout; open-label titration followed by blinded discontinuation.
- **Paired loss.** The mean over replicates of the rejection indicator without dropout minus the rejection indicator with dropout, computed on the same datasets.
- **Analysis half-life $t^a_{1/2}$.** The half-life the analysis uses to build the exposure indicator. It is distinct from the data-generating half-life $t_{1/2}$.

Abbreviations

CAR(1), continuous-time first-order autoregressive; CO, crossover; CR2, bias-reduced cluster-robust variance estimator; MAR, missing at random; MCAR, missing completely at random; MCSE, Monte Carlo standard error; MNAR, missing not at random; OL, open label; OL+BDC, open label plus blinded discontinuation; SE, standard error.

Conflict of interest

The authors declare no conflicts of interest.

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Data availability statement

The data and code that support the findings of this study are in the pmsimstats-ng project repository. Per-replicate outputs and summary files are under `analysis/data/informative-dropout-by-desig` The drivers are under `analysis/scripts/informative-dropout-by-design/`. Exact paths are listed in the Reproducibility section.

Reproducibility

The paired study is produced by `analysis/scripts/informative-dropout-by-design/20-paired-dropout.R` run from the repository root through `bash analysis/scripts/informative-dropout-by-design/20-run-pai` The wrapper sets `VECLIB_MAXIMUM_THREADS`, `OPENBLAS_NUM_THREADS` and `OMP_NUM_THREADS`

to 1. The environment variables `P09_REPS` and `P09_NULL_REPS` set the replicate counts (this manuscript used 1,000 and 500). The driver writes `paired-replicates.rds` (one row per design, regime, replicate, condition and analysis), `paired-dropout.rds` (dropout diagnostics) and `paired-meta.rds` (settings, elapsed time and session information) to `analysis/data/informative-dropout-by-design/`. The script `21-summarise-paired.R` in the same directory writes the four summary files `paired-cells.csv`, `paired-loss.csv`, `paired-contrast.csv` and `paired-dropout.csv`. Every number in Sections 5 to 8 is read from those files by inline R when this manuscript is rendered.

Seeds are `set.seed(20261029 + 100000 * d + r)` for design index d and replicate r, plus 50,000 in the null regime. The run reported here was made on the host R session (R 4.6.1, macOS, outside the project container and without renv). Results from the container are not draw-for-draw identical because the linear-algebra libraries differ. The run took 98 minutes on 8 workers on a shared machine.

The earlier unpaired prototype is `analysis/scripts/quick-sim/09-dropout-prototype-10min.R`. It calls `set.seed(20260508)` once and seeds each replicate with `100000 * i + rep` for cell index i. Its outputs are `analysis/data/quick-sim/09-dropout-replicates.rds` and `09-dropout-summary.txt`. The working protocol `analysis/scripts/informative-dropout-by-design/00-a` and its scaffold drivers (`00-common.R`, `01-study1.R` to `04-study4.R`) were not run.

References

1. Lillie EO, Patay B, Diamant J, Issell B, Topol EJ, Schork NJ. The n-of-1 clinical trial: The ultimate strategy for individualizing medicine? *Personalized Medicine*. 2011;8(2):161-173. doi:10.2217/pme.11.7

2. Hendrickson RC, Thomas RG, Schork NJ, Raskind MA. Optimizing aggregated N-of-1 trial designs for predictive biomarker validation: Statistical methods and theoretical findings. *Frontiers in Digital Health*. 2020;2:13. doi:10.3389/fdgth.2020.00013

3. Matthews JNS, Henderson R. Two-period, two-treatment crossover designs subject to non-ignorable missing data. *Biostatistics*. 2013;14(4):626-638. doi:10.1093/biostatistics/kxt009

4. Blackston JW, Chapple AG, McGree JM, McDonald S, Nikles J. Comparison of aggregated N-of-1 trials with parallel and crossover randomized controlled trials using simulation studies. *Healthcare*. 2019;7(4):137. doi:10.3390/healthcare7040137

5. Vohra S, Shamseer L, Sampson M, et al. CONSORT extension for reporting N-of-1 trials (CENT) 2015 statement. *Journal of Clinical Epidemiology*. 2016;76:9-17. doi:10.1016/j.jclinepi.2015.05.004

548 6. Stunnenberg BC, Berends J, Griggs RC, others, Raaphorst J. N-of-1 trials in neurology: A systematic review. *Neurology*. 2022;98(2):e174-e185. doi:10.1212/WNL.0000000000012998

549 7. Natesan Batley P, McClure EB, Brewer B, et al. Evidence and reporting standards in N-of-1 medical studies: A systematic review. *Translational Psychiatry*. 2023;13(1):263. doi:10.1038/s41398-023-02562-8

550 8. Rubin DB. Inference and missing data. *Biometrika*. 1976;63(3):581-592. doi:10.1093/biomet/63.3.581

551 9. Little RJA, Rubin DB. *Statistical Analysis with Missing Data*. 3rd ed. Wiley; 2019.

552 10. Diggle P, Kenward MG. Informative drop-out in longitudinal data analysis. *Journal of the Royal Statistical Society Series C (Applied Statistics)*. 1994;43(1):49-93. doi:10.2307/2986113

553 11. Little RJA. Pattern-mixture models for multivariate incomplete data. *Journal of the American Statistical Association*. 1993;88(421):125-134. doi:10.1080/01621459.1993.10594302

554 12. Hedeker D, Gibbons RD. Application of random-effects pattern-mixture models for missing data in longitudinal studies. *Psychological Methods*. 1997;2(1):64-78. doi:10.1037/1082-989X.2.1.64

555 13. Wu MC, Carroll RJ. Estimation and comparison of changes in the presence of informative right censoring by modeling the censoring process. *Biometrics*. 1988;44(1):175-188. doi:10.2307/2531905

556 14. Follmann D, Wu M. An approximate generalized linear model with random effects for informative missing data. *Biometrics*. 1995;51(1):151-168.

557 15. Mallinckrodt CH, Clark WS, David SR. Accounting for dropout bias using mixed-effects models. *Journal of Biopharmaceutical Statistics*. 2001;11(1-2):9-21. doi:10.1081/BIP-100104194

558 16. Mallinckrodt CH, Clark SWS, Carroll RJ, Molenberghs G. Assessing response profiles from incomplete longitudinal clinical trial data under regulatory considerations. *Journal of Biopharmaceutical Statistics*. 2003;13(2):179-190. doi:10.1081/BIP-120019265

559 17. Prakash A, Risser RC, Mallinckrodt CH. The impact of analytic method on interpretation of outcomes in longitudinal clinical trials. *International Journal of Clinical Practice*. 2008;62(8):1147-1158. doi:10.1111/j.1742-1241.2008.01808.x

560 18. Molenberghs G, Kenward MG. *Missing Data in Clinical Studies*. Wiley; 2007. doi:10.1002/9780470510445

561 19. Verbeke G, Molenberghs G. *Linear Mixed Models for Longitudinal Data*. Springer; 2000.

562 20. Carpenter JR, Roger JH, Kenward MG. Analysis of longitudinal trials with protocol deviation: A framework for relevant, accessible assumptions, and inference via multiple imputation. *Journal of Biopharmaceutical Statistics*. 2013;23(6):1352-1371. doi:10.1080/10543406.2013.834911

563 21. Cro S, Morris TP, Kenward MG, Carpenter JR. Sensitivity analysis for clinical trials with missing continuous outcome data using controlled multiple imputation: A practical guide. *Statistics in Medicine*. 2020;39(21):2815-2842. doi:10.1002/sim.8569

564 22. Ratitch B, O'Kelly M, Tosiello R. Missing data in clinical trials: From clinical assumptions to statistical analysis using pattern mixture models. *Pharmaceutical Statistics*. 2013;12(6):337-347. doi:10.1002/pst.1549

565 23. Carpenter JR, Kenward MG. *Multiple Imputation and Its Application*. Wiley; 2013.

566 24. O'Kelly M, Ratitch B. *Clinical Trials with Missing Data: A Guide for Practitioners*. Wiley; 2014.

567 25. Permutt T. Sensitivity analysis for missing data in regulatory submissions. *Statistics in Medicine*. 2016;35(17):2876-2879. doi:10.1002/sim.6753

568 26. International Council for Harmonisation of Technical Requirements for Pharmaceuticals for Human Use. ICH E9(R1): Addendum on estimands and sensitivity analysis in clinical trials. Published online 2019.

569 27. Leuchs AK, Zinserling J, Brandt A, Wirtz D, Benda N. Choosing appropriate estimands in clinical trials. *Therapeutic Innovation and Regulatory Science*. 2015;49(4):584-592. doi:10.1177/2168479014567317

570 28. Wulfsohn MS, Tsiatis AA. A joint model for survival and longitudinal data measured with error. *Biometrics*. 1997;53(1):330-339. doi:10.2307/2533118

571 29. Henderson R, Diggle P, Dobson A. Joint modelling of longitudinal measurements and event time data. *Biostatistics*. 2000;1(4):465-480. doi:10.1093/biostatistics/1.4.465

572 30. Berger VW, Rezvani A, Makarewicz VA. Direct effect on validity of response run-in selection in clinical trials. *Controlled Clinical Trials*. 2003;24(2):156-166. doi:10.1016/s0197-2456(02)00316-1

573 31. Morris TP, White IR, Crowther MJ. Using simulation studies to evaluate statistical methods. *Statistics in Medicine*. 2019;38(11):2074-2102. doi:10.1002/sim.8086

574 32. Bell RM, McCaffrey DF. Bias reduction in standard errors for linear regression with multi-stage samples. *Survey Methodology*. 2002;28(2):169-181.